

C15 B EMERGENCY RESILIENCE AND RECOVERY | O (MAX : 4 PT)

Intent : Better enable individuals and communities to maintain health and well-being, and organizations to maintain business function, during and after emergencies.

Summary : This WELL feature requires projects to create a business continuity plan, establish a re-entry plan and offer supportive resources to facilitate resilience during, and recovery after, an emergency.

Issue : Today, communities must prepare for and face emergencies ranging from natural disasters, civil unrest to public health crises. Globally, the frequency, scale and economic and social cost of disasters is increasing due to climate change, population growth and rapid urbanization.¹⁻³ In 2019, the global economic losses from disasters amounted to \$232 billion, and the 2020 COVID-19 pandemic caused the largest global recession in history.^{1,4} Small businesses may be particularly vulnerable as research shows that about 90% of smaller companies fail after emergencies unless they can resume business operations within five days.⁵ Emergencies such as biological events or active shooters may necessitate sheltering in the workplace, while longer-term emergencies may lead to extended workplace shutdowns.⁶⁻¹³ The latter can result in extensive layoffs, while employees who continue working may be forced to work in higher-risk conditions or remotely in spaces not equipped to support productivity.⁹⁻¹³

Solutions : Business continuity planning is critical to help manage business disruption, restore business operations, minimize risk to employees and mitigate financial loss when emergencies occur.¹⁴⁻¹⁷ Establishing organizational remote work readiness can help operations run smoothly and support employee well-being and productivity when an emergency makes remote work imperative.^{14,18} Moreover, employer-funded employee relief assistance can support both employee retention and well-being during and after emergencies.¹⁹⁻²¹ For emergencies that require sheltering on-site, a shelter-in-place plan is crucial to supporting occupant safety.^{6,8,22} Designating space for emergency public use can reduce the burden on medical facilities and help patients receive immediate care.^{23,24} Re-entry plans after emergencies should consider employee needs, offer employees flexibility, assess critical infrastructure systems, communicate re-entry strategies to key stakeholders and re-evaluate existing policies, operations and protocols to support a healthy, safe and inclusive re-entry.²⁵⁻³⁴ In certain types of emergencies, including epidemics and pandemics, re-entry plans may need to consider relevant vaccination, testing, and other requirements (as applicable), including social distancing and use of personal protective equipment in order to help manage risks.³⁵

PART 1 PROMOTE BUSINESS CONTINUITY (MAX : 1 PT)

For All Spaces:

Projects implement a business continuity plan (BCP) that addresses at minimum the following:^{14,18}

- a. Determines critical business functions, processes, supporting resources and dependencies (e.g., email, internet connectivity, third-party suppliers or service providers, interdependent departments).
- b. Includes a list of the roles and responsibilities of the business continuity team and convenes the team annually (at minimum) to review, test and update (as needed) the plan.
- c. Implements a business impact analysis to evaluate the likely effects resulting from disruption of normal business functioning due to a disaster and to identify which critical business functions should be prioritized for recovery.
- d. Conducts a remote work readiness assessment, including at minimum the following:
 1. Evaluates which employees and/or positions (if any) are able to work remotely.
 2. Evaluates which employees and/or positions (if any) have the necessary support infrastructure to work productively in a remote situation.
 3. Evaluates whether organizational technology (e.g., company laptops, virtual private network (VPN)) is set up to support enterprise-wide remote work.
 4. Implements the strategies necessary to support remote work readiness as determined by the evaluation, including (as applicable) methods of communication to employees during remote work and provision for alternate work locations.
- e. Outlines strategies to support short- and long-term continuity in various disasters (e.g., blizzard, pandemic), restore and maintain business operations following disruption and re-mobilize in response to recurring disasters.

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

PART 2 SUPPORT EMERGENCY RESILIENCE (MAX : 1 PT)

For All Spaces:

Projects implement at least one of the following:

- a. Designated outdoor or indoor space is made available to emergency responders, relief organizations or other equivalent institutions at no cost for alternative use in case of emergency (e.g., shelter during a natural disaster, treatment area during a pandemic).
- b. Funding or other resources (e.g., in partnership with local agencies providing relevant services or resources such as vouchers, shelter, clothing, food, transportation) are provided by the employer for emergency use by employees in at least two of the following critical scenarios:
 1. Sheltering from domestic violence or abuse.
 2. Quarantine due to infectious disease exposure.
 3. Damage to employee housing from a disaster.
- c. Shelter-in-place plan for emergencies in which occupants cannot leave the project (e.g., hurricane, chemical spill) that includes the following:
 1. A shelter-in-place kit with resources to help occupants shelter in place within the project for at least 24 hours (e.g., water, food supplies, blankets, flashlights, first aid kit).⁸

2. A process for occupants or groups who may be more vulnerable (e.g., older adults, people with disabilities, pregnant women, children) to confidentially identify specific needs they may have during a shelter-in-place emergency.²²
3. Procedures for communicating to occupants the decision to evacuate or shelter-in-place during an emergency.
4. A commitment to incorporate shelter-in-place guidelines provided by a relevant local-, regional- or global-level emergency response agency (e.g., WHO, government emergency management agency or equivalent) into the plan, and to adhere to instructions provided by that agency during a shelter-in-place emergency.
5. Annual (at minimum) occupant trainings on the shelter-in-place plan.

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PART 3 FACILITATE HEALTHY RE-ENTRY (MAX : 1 PT)

For All Spaces:

Projects establish a plan for re-entry into the project after an emergency (e.g., natural disaster, public health emergency) addressing at minimum the following:

- a. Consultation with regular occupants before and after re-entry to understand their needs and concerns related to re-entry.
- b. Safety, compliance and risk inspections of water, mechanical, electrical, ventilation and life safety systems, including necessary actions to restart building and facility systems after prolonged shutdown and approval or clearance for safe re-entry, as applicable.
- c. A list of roles for those who will be responsible for overseeing the re-entry plan. While roles and contact information should be made available to an organization's personnel, it is not necessary to include this information in the plan submitted for purposes of verifying this feature.
- d. Re-evaluation and adjustment (as needed) of human resources, workplace wellness and employee support policies and amenities (e.g., use of common areas and shared spaces like wellness rooms, food provision, physical activity programs) to support a safer and healthier re-entry.
- e. Policy to support phased re-entry (as needed) offering part-time options, work from home flexibility and/or flexible schedules for all employees (as feasible), particularly for parents and caregivers who may have specific dependencies (e.g., due to childcare closures or a sick family member) and vulnerable groups (e.g., people with disabilities or who may be particularly vulnerable to infectious disease).
- f. Re-evaluation and adjustment (as needed) of facilities management policies and organizational protocols to support safer and healthier re-entry, including but not limited to:
 1. Crowd management and spacing and physical distancing of individuals.
 2. Heightened security measures (e.g., screening, security personnel).
 3. Access to personal protective equipment (PPE).
 4. Additional sanitation supplies and other cleaning or maintenance protocols.
- g. Contingency planning and re-closure measures should the same hazard that forced initial closure re-occur.
- h. Frequent communications through multiple methods (e.g., emails, signage, trainings) to all relevant stakeholders, including (as applicable) employees, occupants, residents, facilities management team, contractors and community members, on the re-entry plan, new or altered policies, operations and procedures, relevant local-, state-, national- or global-level re-entry guidelines and how the project will address occupant health and safety concerns.
- i. Evaluation and incorporation of re-entry guidelines (as available) provided by a relevant local-, regional- or global-level emergency response agency (e.g., WHO, government emergency management agency or equivalent) into the plan, and adherence to instructions provided by that agency during re-entry.

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PART 4 ESTABLISH HEALTH ENTRY REQUIREMENTS (MAX : 1 PT)

For All Spaces except Dwelling Units:

Projects located in a region with heightened risk of infectious respiratory disease transmission defined by a public health authority (e.g., World Health Organization, local public health agency) require at least one of the following for regular occupants to enter the space:

- a. Proof of vaccination or documented exemption (e.g., a medical or religious exemption), including necessary

boosters as applicable, for the disease of heightened risk.

b. Both of the following:

1. Proof of negative diagnostic testing for the disease of heightened risk and, for eligible employees (as applicable), access to diagnostic testing at no cost.
2. Face masks worn indoors by, at minimum, unvaccinated occupants. Face masks are available for all occupants.³⁶

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